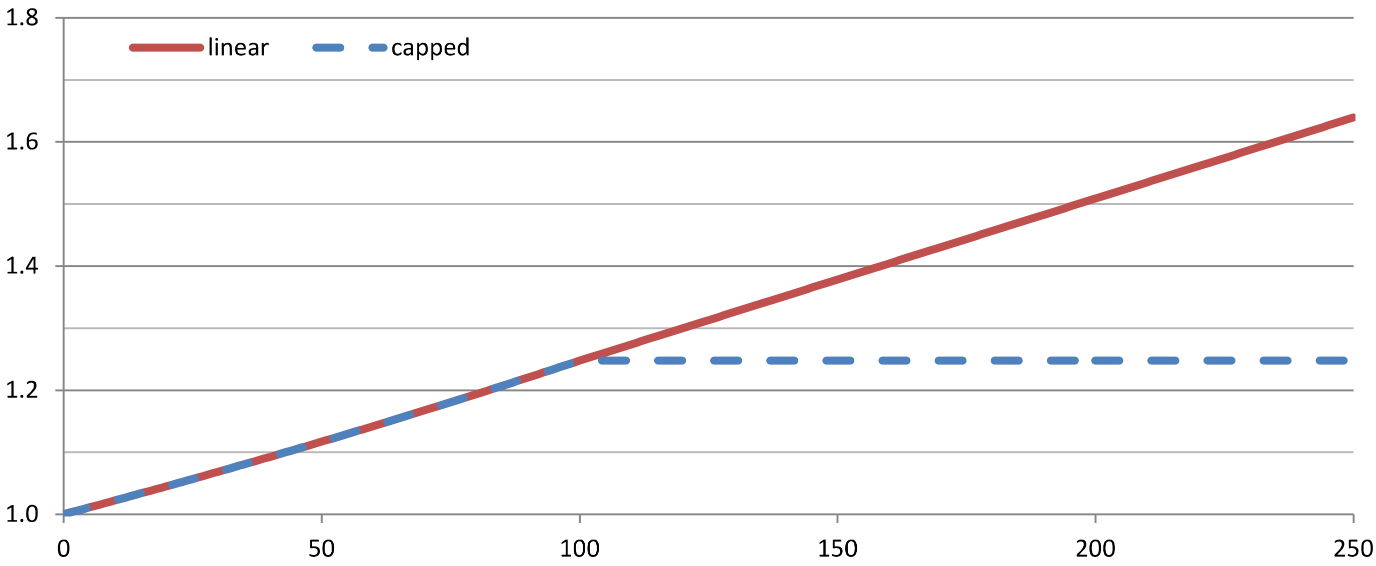
(5).(2) Ischaemic heart disease mortality, men (two options)

Condition category:	**(5) Cardiovascular conditions**
ICD10 codes:	**I20 to I25**

	Current drinkers	**Former drinkers**
Source	Zhao et al. (2017) Table 3, top panel, fully-adjusted results, custom analysis (see comments)	Roerecke & Rehm (2010b) Table 3
Relative risk Function or estimate	$\ln RR(x) = 0.002211x$ $RR(x)$=exp($0.002211x$)	RR_{FD} = 1.25
Comments	Fully-adjusted model for the younger age cohort in Table 3 was re-analyzed by the first author upon request to include a gender breakdown and to provide a continuous relationship. Results received through personal correspondence between AS, TS and J. Zhao (dated 14-Oct-17).	Results stratified by gender and endpoint (outcome) used.
Control for abstainer bias Does the article control for abstainer bias? If so, how?	Yes. This study explicitly controlled for abstainer biases selecting studies with no bias and selecting younger cohorts, among other methods. See article for full methodology.	Yes, considered. The reference group was operationalized as "long-term abstainers or very light drinkers."



Sources

Zhao, J., Stockwell, T., Roemer, A., Naimi, T., Chikritzhs, T. (2017). Alcohol consumption and mortality from coronary heart disease: An updated meta-analysis of cohort studies. *Journal of Studies on Alcohol and Drugs, 78*, 375-386.

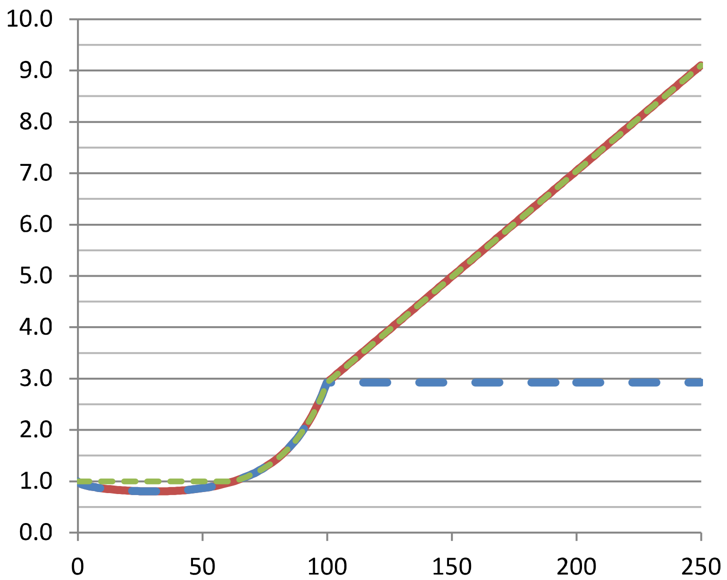
Roerecke, M., & Rehm, J. (2010b). Ischemic heart disease mortality and morbidity rates in former drinkers: a meta-analysis. *American journal of epidemiology*, *173*(3), 245-258.

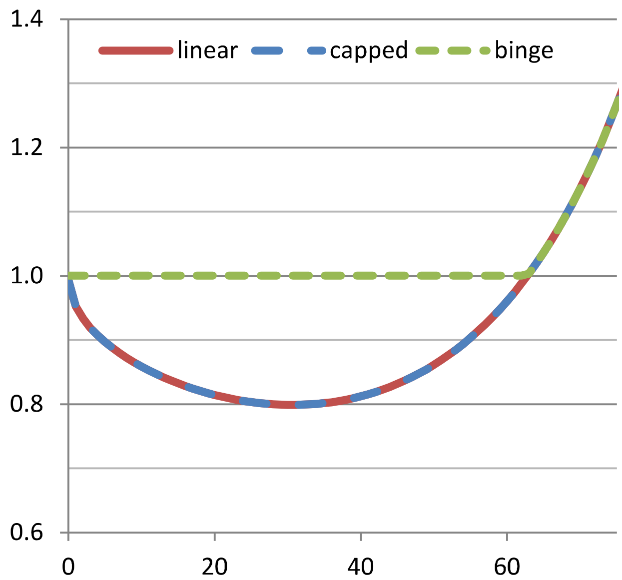
(5).(2) Ischaemic heart disease mortality, men (two options)

Condition category: (5) Cardiovascular conditions

ICD10 codes: I20 to I25

	Current drinkers	**Former drinkers**
Source	Roerecke & Rehm (2012) Figure 2 Roerecke & Rehm (2010a) From text, e.g. in abstract	Roerecke & Rehm (2010b) Table 3
Relative risk Function or estimate	$\ln RR(x) = -0.04870068\sqrt{x} + 0.000001559x^3$ $RR(x)=\exp(-0.04870068\sqrt{x} + 0.000001559x^3)$	$RR_{FD} = 1.25$
Comments	Relative risk function received directly from members of authorship group who are members of this project. Roerecke & Rehm (2010a) modifies the RR curve for bingers by removing the protective effect (i.e. RR=1.0).	Results stratified by gender and endpoint(outcome) used. Note: In the figure below, the binge level is set at 60g/day; therefore RR=1.0 above this as this portion of the population is guaranteed to binge.
Control for abstainer bias Does the article control for abstainer bias? If so, how?	Yes. This study reweighted relative risk results from studies which pooled former and never drinkers as abstainers using a standard methodology.	Yes, considered. The reference group was operationalized as "long-term abstainers or very light drinkers."





Sources

Roerecke, M., & Rehm, J. (2012). The cardioprotective association of average alcohol consumption and ischaemic heart disease: a systematic review and meta-analysis. *Addiction*, *107*(7), 1246-1260.

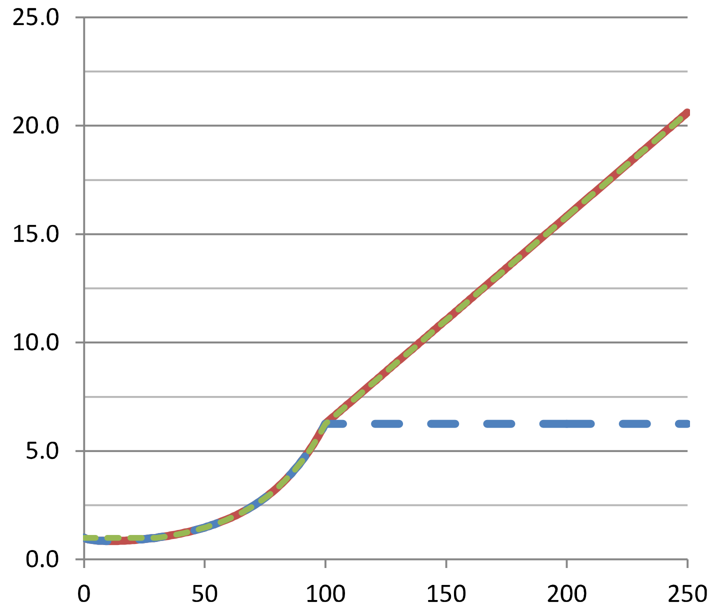
Roerecke, M., & Rehm, J. (2010a). Irregular heavy drinking occasions and risk of ischemic heart disease: a systematic review and meta-analysis. *American journal of epidemiology*, *171*(6), 633-644.

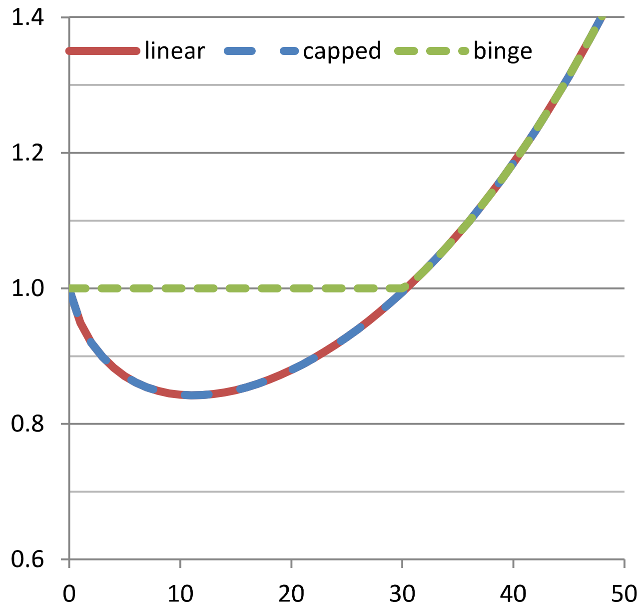
Roerecke, M., & Rehm, J. (2010b). Ischemic heart disease mortality and morbidity rates in former drinkers: a meta-analysis. *American journal of epidemiology*, *173*(3), 245-258.

(5).(2) Ischaemic heart disease mortality, women

Condition category: (5) Cardiovascular conditions
ICD10 codes: I20 to I25

	Current drinkers	Former drinkers
Source	Roerecke & Rehm (2012) Figure 2 Roerecke & Rehm (2010a) From text, e.g. in abstract	Roerecke & Rehm (2010b) Table 3
Relative risk Function or estimate	$\ln RR(x) = -0.0525288x + 0.0153856x \ln x$ $RR(x)=\exp(-0.0525288x + 0.0153856x \ln x)$	$RR_{FD} = 1.54$
Comments	Relative risk function received directly from members of authorship group who are members of this project. Roerecke & Rehm (2010a) modifies the RR curve for bingers by removing the protective effect (i.e. RR=1.0).	Results stratified by gender and endpoint (outcome) used. **Current**: In the figure below, the binge level is set at 60g/day; therefore RR=1.0 above this as this portion of the population is guaranteed to binge.
Control for abstainer bias Does the article control for abstainer bias? If so, how?	Yes. This study reweighted relative risk results from studies which pooled former and never drinkers as abstainers using a standard methodology.	Yes, considered. The reference group was operationalized as “long-term abstainers or very light drinkers.”





Sources

Roerecke, M., & Rehm, J. (2012). The cardioprotective association of average alcohol consumption and ischaemic heart disease: a systematic review and meta-analysis. *Addiction*, *107*(7), 1246-1260.

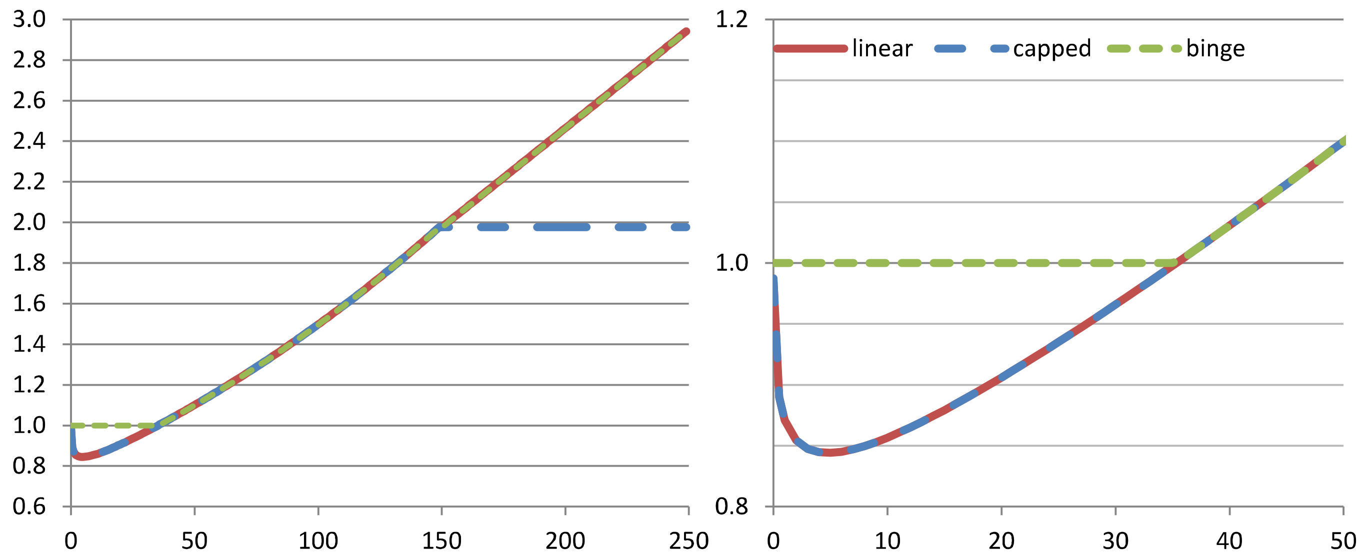
Roerecke, M., & Rehm, J. (2010a). Irregular heavy drinking occasions and risk of ischemic heart disease: a systematic review and meta-analysis. *American journal of epidemiology*, *171*(6), 633-644.

Roerecke, M., & Rehm, J. (2010b). Ischemic heart disease mortality and morbidity rates in former drinkers: a meta-analysis. *American journal of epidemiology*, *173*(3), 245-258.

(5).(6) Ischaemic stroke mortality, men

Condition category:	**(5) Cardiovascular conditions**
ICD10 codes:	**I63 to I67, I69.3**

	Current drinkers	**Former drinkers**
Source	Patra et al. (2010) Figure 7 Rehm et al. (2016) From text, in methods	Larsson et al. (2016) Supplementary Table S2, pooled analysis
Relative risk Function or estimate	$\ln RR(x) = -0.1382664\sqrt{x} + 0.03877538\sqrt{x}\ln x$ $RR(x)=\exp(-0.1382664\sqrt{x} + 0.03877538\sqrt{x}\ln x)$	$RR_{FD} = 0.97$
Comments	Relative risk function received directly from members of authorship group who are members of this project. Rehm et al. (2016) modifies the RR curve for bingers by removing the protective effect (i.e. RR=1.0).	Note: In the figure below, the binge level is set at 60g/day; therefore RR=1.0 above this as this portion of the population is guaranteed to binge.
Control for abstainer bias Does the article control for abstainer bias? If so, how?	Yes. This study reweighted relative risk results from studies which pooled former and never drinkers as abstainers using a standard methodology.	No. This study does not explicitly account for abstainer bias.



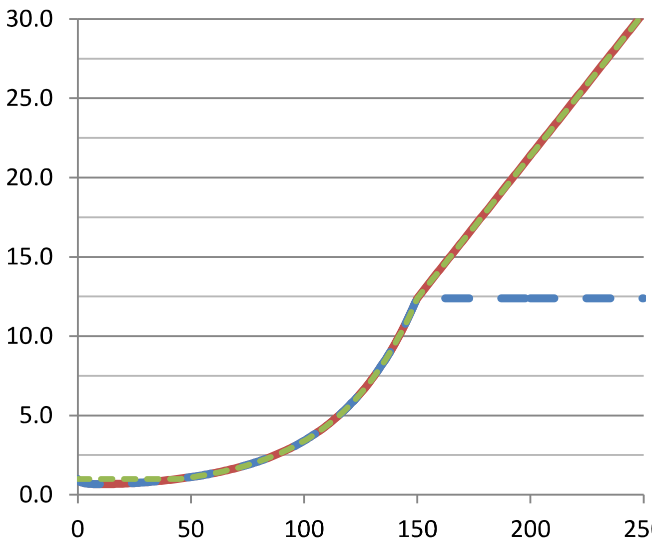
Sources

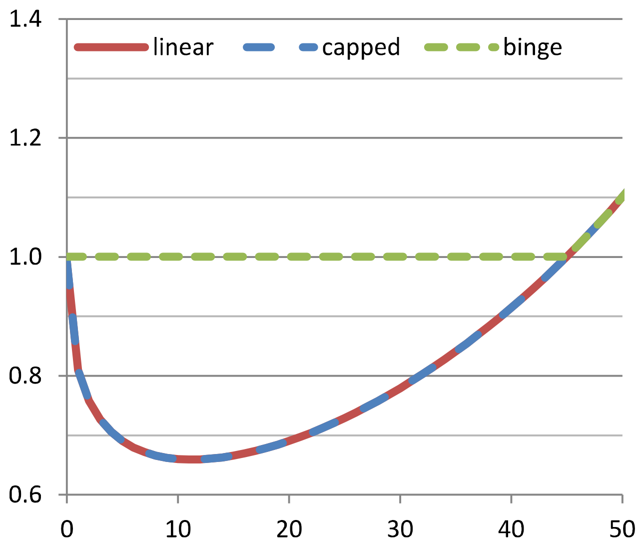
Patra, J., Taylor, B., Irving, H., Roerecke, M., Baliunas, D., Mohapatra, S., & Rehm, J. (2010). Alcohol consumption and the risk of morbidity and mortality for different stroke types-a systematic review and meta-analysis. *BMC public health*, *10*(1), 258.
Larsson, S. C., Wallin, A., Wolk, A., & Markus, H. S. (2016). Differing association of alcohol consumption with different stroke types: a systematic review and meta-analysis. *BMC medicine*, *14*(1), 178.
Rehm, J., Shield, K. D., Roerecke, M., & Gmel, G. (2016). Modelling the impact of alcohol consumption on cardiovascular disease mortality for comparative risk assessments: an overview. *BMC Public Health*, *16*(1), 363.

(5).(6) Ischaemic stroke mortality, women

Condition category: (5) Cardiovascular conditions
ICD10 codes: I63 to I67, I69.3

	Current drinkers	Former drinkers
Source	Patra et al. (2010) Figure 7 Rehm et al. (2016) From text, in methods	Larsson et al. (2016) Supplementary Table S2, pooled analysis
Relative risk Function or estimate	$\ln RR(x) = -0.248768\sqrt{x} + 0.03708724x$ $RR(x)=\exp(-0.248768\sqrt{x} + 0.03708724x)$	$RR_{FD} = 0.97$
Comments	Relative risk function received directly from members of authorship group who are members of this project. Rehm et al. (2016) modifies the RR curve for bingers by removing the protective effect (i.e. RR=1.0).	Note: In the figure below, the binge level is set at 60g/day; therefore RR=1.0 above this as this portion of the population is guaranteed to binge.
Control for abstainer bias Does the article control for abstainer bias? If so, how?	Yes. This study reweighted relative risk results from studies which pooled former and never drinkers as abstainers using a standard methodology.	No. This study does not explicitly account for abstainer bias.





Sources
Patra, J., Taylor, B., Irving, H., Roerecke, M., Baliunas, D., Mohapatra, S., & Rehm, J. (2010). Alcohol consumption and the risk of morbidity and mortality for different stroke types-a systematic review and meta-analysis. *BMC public health*, *10*(1), 258.
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